

Mohr Insurance — Dynamic Sales Script

Script version 10 · branched reference & agent worksheet · one needs assessment for every client · employer and T65 education · umbrella priced from the needs assessment

Standing compliance rule

BEFORE ANYTHING ELSE

A client who is 65 or older in California cannot be pitched ancillary — just pitch Medicare. Cancer / heart attack / stroke cover and skilled nursing cover cannot be sold, so **do not embed either exposure** — skip the major-exposures set-up, the CHS conversation and the skilled nursing conversation entirely.

Dental can still be sold. Ask whether it is important to them as normal; it still prices \$50 into the Medicare Supplement umbrella. That leaves the Advantage umbrella at \$0, since an Advantage plan already bundles dental.

Gold / Silver / Bronze is cancer, heart attack and stroke cover, so it cannot be sold either. On the employer track there is no Medicare plan to offer in its place, so that call ends after the delay recommendation and the CMS L564 education.

Their zip code and date of birth decide all of this, and both are the first two questions of the needs assessment.

How this script branches

EVERY CALL

- |
- | — 1. Opening — greeting · recording · Medicare disclaimer
 - | — "Enrolled in Parts A & B?" — YES —► 4. ALREADY ON MEDICARE — paused
 - | — "Make your own healthcare decisions?" — NO —► STOP, get the POA on the line
 - | — Scope of Appointment — NO —► STOP, cannot present
 - | — Goals loop (repeat until "nothing else")
- |
- | — 2. CNA — the full assessment, every question, for every client below
 - | zip · DOB · work status ("over 20 employees?")
 - | SS · carrier · HMO/PPO · premium · deductible/MOOP
 - | hit it? + health history · Rx
 - | DENTAL* · CHS*† · SKILLED NURSING*† · Part B income · assets
 - |
 - | over 20 employees —► 3. EDUCATION (EMPLOYER)
 - | (20 or fewer → must transition) Rec 1: delay Part A and Part B
 - |
 - | penalties · CMS L564 (the hall pass)
 - | Rec 2: max OOP → risk → Gold/Silver/Bronze

explain the why → commitment → address

5. MEDICARE EDUCATION — T65

"Familiar with the two? Leaning one way or the other?"

└ leaning Supplement —► A & B summary → 6. Supplement

└ leaning Advantage —► A & B summary → 7. Advantage

└ not familiar —► A & B summary → paper → 6 → 7

→ 8. Price (both sides, or their side if leaning) → leaning / confirm

→ formal recommendation → Scenario A (not on SS) / Scenario B (on SS)

→ underwriting urgency → address → objections + pre-emptive close

* umbrella drivers (T65 track) † skipped entirely when the client is 65+ in California

Gold / Silver / Bronze is the product on the employer track.

The \$50/\$100/\$150 umbrella built from the needs assessment is T65 only.

Track	How you land on it	Education	Close
Employer coverage	Not on Medicare · still working · over 20 employees . Runs the full needs assessment first.	Delay Part A and Part B · L564 · Gold / Silver / Bronze	Commitment, then address
T65 — not on SS	Not on Medicare, not drawing Social Security	Supplement and / or Advantage, by how they lean	Scenario A
T65 — on SS	Not on Medicare, drawing Social Security	Supplement and / or Advantage, by how they lean	Scenario B
Already on Medicare	Parts A & B active	Paused — this script is being rewritten. Capture the basics and book a follow-up.	

*An employer plan with **20 or fewer employees** makes Medicare the primary payer — those clients **must transition to Medicare**, so they run the T65 track.*

Umbrella pricing — T65 track only

*Clients on employer coverage are sold **Gold / Silver / Bronze** instead. This table does not apply to them.*

What they told you in the CNA	Medicare Supplement	Medicare Advantage
Cancer / heart attack / stroke is a concern	+\$50	+\$50
Skilled nursing is a concern	+\$50	+\$100
Dental is important to them	+\$50	included with the plan
Maximum umbrella	\$150	\$150
California, 65 or older — cancer and skilled nursing removed, dental only	\$0 or \$50	\$0

Quote	Formula
Medicare Supplement	Plan G premium + Part D $\approx$ \$10 + umbrella (\$50 / \$100 / \$150) = one monthly price
Medicare Advantage	MAPD premium (usually \$0) + umbrella (\$50 / \$100 / \$150) = one monthly price

Quote the single all-in number. Do not itemize it out loud unless they ask.

65 or older in California: only the dental component survives. The Supplement umbrella is \$50 if dental matters to them and \$0 if it does not; the Advantage umbrella is \$0 either way. When the umbrella lands at \$0 there is nothing to sell today, and the close drops to two steps.

1 · Opening

Greeting and webinar confirmation

"Hey [Name], this is [Agent name] with Mohr Insurance Services calling for our scheduled Medicare appointment... how's it going today?"

Wait for their response.

"Looks like you made it through our webinar?"

Wait for the response.

IF THEY MADE IT THROUGH

"Awesome, and that webinar is designed to give you a general overview of what your transition to Medicare looks like. On this call we'll dive into your specific situation."

IF THEY DIDN'T MAKE IT THROUGH

"No worries, that webinar is designed to give you a general overview of what your transition to Medicare looks like. On this call we'll dive into your specific situation."

Call recording disclosure

"Now [Name], before we continue, I have to read these compliance disclosures. You know we take compliance very seriously, so I need to let you know that **this call is being recorded for quality and compliance purposes.**"

Required Medicare disclaimer

"We do not offer every plan available in your area. Any information I provide is limited to the plans we represent. You can always visit Medicare.gov or call 1-800-MEDICARE to review all available options."

"Now, almost done here!"

Required verbatim. Do not paraphrase or skip.

Eligibility check branch point

"And are you currently enrolled in Medicare Parts A and B?"

IF YES

They are **already on Medicare**. That script is **paused** while it is rewritten — finish the opening and the goals loop, then see section 4.

"And do you personally make your own healthcare decisions?"

IF NO

Stop. You cannot present or enroll without the authorized representative or power of attorney on the line. Ask who handles their healthcare decisions and get that person on the call, or reschedule with them present.

Scope of Appointment permission

"Now for today's call, we may discuss: Medicare Advantage Plans · Medicare Supplement Plans · Prescription Drug Plans · Ancillary Products. Do I have your permission to discuss these options with you today?"

*Wait for a **clear verbal YES**.*

IF NO

Do not proceed. Without a Scope of Appointment on the recording you cannot discuss any of the four product categories.

Transition into the CNA — the goals loop

"Perfect, now that we got through those compliance disclosures, **do you have any specific questions or anything specific you want to accomplish today?**"

Write down what they say — their words.

1.

2.

3.

"Perfect, so what I'm hearing is (repeat their goal). Does that sound about right?"

"Perfect, is there anything else you want to go over today?"

Loop until they have nothing else. You read every one of these back in the formal recommendation.

PARTS A & B ALREADY ACTIVE

Skip the needs assessment below and go to **section 4**.

Transition into the needs assessment

"Perfect. So what I'd like to do now is take you through what's called a **Client Needs Assessment**. That way I can better understand your situation, and by the end of the call I'll be able to make a recommendation for you. **Is that okay with you?**"

Wait for their yes.

2 · Client Needs Assessment

Every client who is not already on Medicare answers **every question** in this section — employer clients included.

Zip code

"So for starters, what's your zip code?"

Repeat the zip code back. → "Awesome, got that down."

Zip:

Date of birth

"Now what is your date of birth?" → "Perfect, got that here."

DOB (mm/dd/yyyy): Age: Birth month:

Work status branch point

"Tell me a little bit about your current situation — are you still working, are you retired, what does that look like?"

IF WITH AN EMPLOYER

"Does your employer have over 20 employees?"

Over 20 → employer track. **Finish the whole needs assessment**, then go to section 3, Education (Employer).

20 or fewer → they must transition to Medicare. Finish the needs assessment, then the T65 education.

Work status:

Social Security status branch point — T65

"Are you drawing Social Security yet?"

YES

Automatically enrolled into A & B · **Scenario B** at close.

NO

We enroll them manually · **Scenario A** at close.

On the employer track this is captured for the record only — the recommendation is to delay Part A and Part B either way.

Current insurance carrier

"Tell me a little bit about your current health insurance. What carrier is that with, and how has it been treating you?"

"And is that an HMO... PPO?"

Carrier: HMO / PPO: How it's treating them:

Monthly premium

"What are you paying for that right now on a month to month basis?"

\$ /mo

Deductible and max out of pocket

"And then you may not know this next part off the top of your head, but if you have your insurance card it should read off of there. What is your deductible and maximum out of pocket?"

Deductible: \$ Max out of pocket: \$

The max out of pocket is the entire basis of the employer-track pitch. Do not leave this blank.

Health history

"And that leads me into my next question — have you ever hit that deductible or max out of pocket? Anything in your health history we should know about in the past 5 years?"

Underwriting matters here.

"And are you taking any prescriptions right now, and what are they actively treating?"

Dental importance

umbrella driver

still asked in CA at 65+

"My next question would be, is dental insurance important to you, and do you currently have that with your employer or through the marketplace?"

IF YES — T65

+\$50 to the Medicare Supplement umbrella. No effect on the Advantage umbrella — an Advantage plan already bundles dental.

Major exposures — the set-up

skipped in CA at 65+

"Now in that webinar, do you remember how we highlight major exposures?"

"Major exposures can really stem from any plan — whether it's like your current plan with [carrier], or a Medicare Supplement or Advantage plan — but really in relation to two specific things."

Cancer, heart attack, stroke

umbrella driver

skipped in CA at 65+

"For starters — cancer, heart attack, stroke. Do we have any personal or family history with the big three?"

"I appreciate you sharing that with me... and the reason we bring this up is because there's always out of pocket expenses that come along with a diagnosis for cancer, heart attack and stroke. **I'm assuming you don't want to be exposed to those out of pocket expenses, right?**"

IF YES — T65

+\$50 to both umbrellas.

Skilled nursing

umbrella driver

skipped in CA at 65+

"Now the next big exposure we see is skilled nursing... do you have any personal or family history with that, and are you familiar with how that coverage functions with Medicare?"

"Really what you need to know is Medicare will help cover up to day 100 as long as you had a **3 day hospital stay** and are **showing signs of improvement**... but after day 100 you are responsible for all costs. **So I'm assuming this will be a concern as well?**"

IF YES — T65

+\$50 to the Supplement umbrella · +\$100 to the Advantage umbrella.

"And yeah, we'll come back to that in greater detail, but I really just wanted to make sure you're aware of these exposures."

Part B premium — income qualification

"Now, when you transition to Medicare you'll be responsible for a Part B premium — however that premium is going to be very dependent on your household income. Do you know where that's at at this moment in time?"

"And has that been pretty consistent over the last two years?" ... "And the reason I ask is they always look back two years prior."

Income: Consistent 2 yrs:

Retirement assets

"And another thing that can impact your Part B premium, which not a lot of people think about — do you have any retirement accounts that have over \$200,000 in qualified assets or more?"

"And the reason I ask is because when we plan to supplement our income with the money within those accounts, that can impact your Part B premium. Have you had the chance to talk to a retirement specialist yet?"

IF NO

"Okay, well we have a full partnership on hand, so if you ever want to sit down and have that conversation it is a service we offer **free of charge**, and we can look at getting you scheduled for that once we're done here — okay?"

Close out the CNA

"Awesome, that wraps up the needs assessment. Now we're going to walk through your options and answer the questions you had at the beginning."

3 · Education (Employer)

Employer coverage with over 20 employees, after the full needs assessment. This track ends here: no Supplement, no Advantage, no umbrella.

Recommendation #1 — delay Medicare

"So right now this transition to Medicare does not make much sense for you, so my first recommendation would be to **delay Part A and Part B entirely**. You have a strong plan with your employer and your premiums are pretty low."

Peace of mind on penalties

"And I want to make sure you are at peace of mind with penalties, since that's always a big concern. **You won't be penalized for delaying Medicare** since you have creditable coverage with your employer. Are we clear on that?"

Wait for the response. Do not move on without a yes.

CMS L564 — the hall pass

"Now when you do decide to retire, or you lose that creditable coverage, that's when you take action. That's when you take **CMS L564** — it's a form you'll take to your HR department and they'll help you fill it out. Then you'll take that form and you're gonna submit that form along with your application for **Part A and Part B**. That form is basically your **hall pass that voids all penalties**. Are we clear on that?"

CALIFORNIA, 65 OR OLDER — STOP HERE

Gold / Silver / Bronze is cancer, heart attack and stroke cover, so it cannot be sold, and there is no Medicare plan to offer in its place because they are staying on employer coverage. **End the call as education**. Answer their questions, make sure they know to call when they lose creditable coverage, and note when to follow up.

Recommendation #2 — cover the gaps

"Now my second recommendation would be to protect you from the gaps that employer plans typically leave behind."

STEP 1 — REITERATE THEIR EXPOSURE: MAX OOP

"Because right now, your employer plan has a max out of pocket of **[max OOP]**."

STEP 2 — EXPLAIN THE RISK

"What this means is, god forbid you have a bad health year, it's very likely you will pay [**max OOP**] for your healthcare **plus** potentially have loss of income."

STEP 3 — PRESENT GOLD / SILVER / BRONZE

"That's where our **Gold, Silver and Bronze** options come in — they're designed to cover those big out-of-pocket risks while you stay on your employer plan."

"Now [first name], I want you to write down Gold, Silver and Bronze, and let me know once you've got that down."

*Wait for confirmation before **each** tier below. One at a time — this pace is the whole close.*

Tier	Cancer	Heart attack & stroke	Total benefit	Monthly
Gold	\$30,000	\$30,000	\$60,000	\$150
Silver	\$20,000	\$20,000	\$40,000	\$100
Bronze	\$10,000	\$10,000	\$20,000	\$50

Explain the why

"So the reason we're even looking at this today is for one, you have your **health and your age working for you**, so not only do you have the best chance to qualify for this type of coverage, but assuming that you qualify, you have the opportunity to **lock in the cheapest rate** you can qualify for. Once you lock in that rate it does not change."

Once they confirm:

"So based on these prices, which option do you feel like you can comfortably afford on top of your employer coverage?"

***Get the commitment**, then move forward with the application.*

"Sounds good, so what would be a good address to send policy packets and information?"

*Handle objections and **always end with a pre-emptive close**.*

4 · Already on Medicare — paused

There is no script for clients already on Medicare in this version — it is being rewritten. Don't improvise a recommendation.

Capture their zip, date of birth, carrier and what they have (Advantage or Supplement, plan, premium, what prompted the call). Book a follow-up and wrap the call.

Zip: DOB: Carrier:

What they have / want:

Follow-up booked for:

5 · Medicare Education — T65

"So in the webinar we talk about the difference between Medicare Supplement and Medicare Advantage. Are you familiar with the two? Are you leaning towards one way or the other? **What are your thoughts?**"

Wait for the response.

LEANING TOWARD MEDICARE SUPPLEMENT

Part A and B summary, then **section 6 only**, then pricing and the recommendation.

LEANING TOWARD MEDICARE ADVANTAGE

Part A and B summary, then **section 7 only**, then pricing and the recommendation.

NOT FAMILIAR

Continue with every step below. Tell them you'll cover both:

"Now we'll go over both today."

Two-sentence summary of Part A and Part B

"So for starters you have Part A and Part B. **Part A is premium free** and **Part B** is going to have that monthly premium we talked about."

"Now with just Part A and Part B you are left with that **big hospital deductible** and that **20% coinsurance** you would be responsible for with **no maximum out of pocket**."

NOT FAMILIAR — TRANSITION TO SUPPLEMENT AND ADVANTAGE

"So what we do is, we can look at a supplement or an advantage plan to help fill those gaps."

Pen and paper

"Do you have a piece of paper and pen with you?"

Wait for the response. They need it for the 3 simple steps at the close, whichever way they lean.

NOT FAMILIAR

"What I want you to do is put a line down the middle of the paper. On the left put **supplement** and on the right put **advantage**, and we are going to start on the left side."

6 · Medicare Supplement education

"So for Medicare Supplement, you are going to have an **additional premium on top of your Part B premium**, but it is the more comprehensive option. You don't have networks, you don't need prior authorizations, you don't need referrals — it gives you the **flexibility and freedom** to go where you want."

"You'll have an annual deductible of **\$283**, and once you hit that deductible all other Medicare-approved expenses are covered at 100%. For all non-Medicare approved expenses, that's what the **umbrella coverage** is for, and we'll get to that in more detail very soon."

California and 65 or older: leave out the umbrella sentence — the umbrella can't cover non-approved expenses there.

"And if dental is important to you, that will be a separate premium, and you'll need a separate prescription drug plan as well."

"So to recap Medicare Supplement: you'll be responsible for an additional premium, you'll have the freedom to go where you want when you want, and your Medicare-approved expenses will be very predictable."

"Now do you have any questions on how a Medicare Supplement functions?"

7 · Medicare Advantage (Part C) education

"Now for Medicare Advantage, it's going to be more like your **all in one plan**, so not only is it going to include your health insurance, but it will also include **dental, vision, hearing and prescription drug coverage**."

"Now you can have lower monthly premiums with an advantage plan, but you will have **copays for doctor visits** and **coinsurance for major procedures**, so that's why we never recommend it without umbrella coverage."

California and 65 or older: drop "so that's why we never recommend it without umbrella coverage" — there is no Advantage umbrella to sell there.

"And if this is the route you take, you have to be comfortable with **prior authorizations**, being **in a network**, and needing **referrals** to see specialists. We like to think 9 times out of 10 Medicare Advantage is a **better version of your current plan**, because these plans usually have little to no deductible and very affordable copays."

"Any questions on how a Medicare Advantage plan functions?"

8 · Pricing, recommendation and close

Work out the price **before** you start talking cost. Not familiar: price both sides. Leaning one way at the start of education: price only that side.

Piece	Medicare Supplement	Medicare Advantage
Supplement (Plan G premium)		—
PDP	≈ \$10	—
MAPD (usually \$0)	—	
CHS — if cancer / heart attack / stroke is a concern	\$50	\$50
RC — if skilled nursing is a concern	\$50	\$100
Dental — if dental matters to them	\$50	included with the plan
Total		

California and 65 or older: CHS and RC can't be sold — leave them out. Dental is the only add-on left, on the Supplement side only.

"Now that we've gone over the benefits of both plans, let's talk about cost."

"For your **Medicare Supplement** [with umbrella], this will be **\$(total) a month.**"

"For your **Medicare Advantage** [with umbrella], this will be **\$(total) a month.**"

Say "with umbrella" only when there is umbrella coverage in that price (CHS, RC or Dental). Leaning one way: say only their side.

Which option are they leaning toward?

"Now that we've gone through both options and the detailed pricing breakdown, which route do you feel yourself leaning toward as of now?"

PAUSE. Let them answer. If they leaned one way at the start of education and only heard that price, skip the question and just confirm.

"Okay, so you're leaning toward [Medicare Advantage / Medicare Supplement]?"

Get their confirmation again.

Formal recommendation

"Can I make a recommendation for you?"

Wait for their yes before you go into the recommendation.

The interactive script writes this rationale for you from their goals and the needs assessment. On paper, build it in this order:

"Based on everything you shared today — you told me what you wanted out of today: **[their goals, in their words]**."

"That's why the **[Medicare Supplement / Medicare Advantage]** makes sense for you: *[Supplement: it's the more comprehensive option — no networks, no prior authorizations, no referrals — and once you meet the \$283 deductible, your Medicare-approved expenses are covered. / Advantage: it's your all-in-one plan — your health coverage, your prescriptions, and dental, vision and hearing together, usually for a \$0 premium.] [Tie it to any goal about doctors, cost, prescriptions or understanding their options.]"*

"*[Only what they selected:]* You also shared your history with cancer, heart attack and stroke, and that you don't want to be exposed to those out-of-pocket costs — that's what the cancer, heart attack and stroke coverage is for. And since Medicare stops paying for skilled nursing after day 100, the recovery care covers that gap. Dental matters to you, so dental coverage is included as well."

"So my formal recommendation would be the **[Supplement / Advantage] plan**, with umbrella coverage for about **[\$[X]]** a month."

PAUSE. The first one to talk loses.

The 3 simple steps branch point

"Do you still have that piece of paper? Perfect — I want you to write this down, because this is how we will get everything done, in these 3 simple steps."

SCENARIO A — T65, NOT ON SOCIAL SECURITY

1. We get your umbrella coverage set up for \$[X] per month, and that will be for your **[cancer, heart attack and stroke coverage / recovery care / dental coverage — whichever they selected]**.
2. We manually enroll you into Original Medicare Parts A & B three months before your birth month — it takes about 10 minutes.
3. We enroll you into your **[Supplement — 6 months / Advantage — 3 months]** before your 65th birthday.

SCENARIO B — T65, ON SOCIAL SECURITY

1. We get your umbrella coverage set up for \$[X] per month, and that will be for your **[cancer, heart attack and stroke coverage / recovery care / dental coverage — whichever they selected]**.
2. Since you're on Social Security you'll be automatically enrolled into Parts A & B — we just confirm that's all set.
3. We enroll you into your **[Supplement — 6 months / Advantage — 3 months]** before your 65th birthday.

California and 65 or older with no dental sale: step 1 is removed and this becomes a two-step close. If dental matters to them, step 1 is the \$50 dental umbrella on the Supplement side.

Why we do this today

"The reason we want to go ahead and handle this now is because we need to make sure we can get you **approved through underwriting**. That way, if for some reason one company doesn't approve of you, we still have time to pivot and find another option before your effective date."

"The other important part is once you're approved, this **locks in your rate based on your age right now** — so it's never going to jump because of your health later on."

"I'll handle the entire enrollment process for you. It's only a few quick health questions and we should be able to get everything wrapped up in the next few minutes."

"Now let me get everything pulled up for us. And what was going to be a good street address for you, so I make sure you receive your packages and cards?"

*Handle objections and **always end with a pre-emptive close.***

